THE MATERNITY METHOD

NCLEX CHEAT SHEET

Preterm Labor & Delivery

Maternal & Newborn › Labor & Delivery

The Big Picture **Preterm labor (PTL)** = contractions that **change the cervix** between **20 and 37 weeks**. Our job: **keep baby inside** as long as it's safe and **get baby's lungs ready**. More days in the womb = fewer newborn problems.

KNOW THE NUMBERS

< 37 wks	< 34 wks	< 32 wks	RR < 12	1 hour
Born before this = **preterm**	Give **betamethasone** for baby's lungs	**Mag** protects baby's brain	**Stop** the magnesium	Rest + fluids, then **call** if still contracting

Spot It

- **Regular contractions**: feels like period cramps or belly tightening
- **Low backache** that is dull and won't go away
- **Pelvic pressure**: "baby is pushing down"
- **Discharge change**: more, watery, mucus, or blood-tinged
- **Cervix thinning or opening** on exam

Signs can be **mild** and look like normal pregnancy aches, so teach **every** patient what to watch for.

Who's at Risk

- **Past preterm birth**: the strongest predictor
- **Stretched uterus**: twins or more, too much amniotic fluid
- **Infection**: UTI, STIs, BV, gum disease
- **Short cervix** on ultrasound
- **Smoking** or **substance use**
- **Little or no prenatal care**

MEMORY HACK · HOME TEACHING

Contractions before 37 weeks? Use the 4 S's

Stop	Side	Sip	Sixty
what you're doing	lie down on your side	drink fluids	still going after 1 hr? Call the provider

Prevention = catch risks early. Keep every prenatal visit · screen and treat infections · help with quitting smoking and substances · high-risk patients get **more frequent visits**.

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Nursing Actions

Goal: **slow labor** + **get baby ready**

STEP 1 — Assess

- **Cervical exam** for thinning (effacement) and opening (dilation)
- **Fetal fibronectin (fFN) swab: negative = reassuring**, birth in the next 7–14 days is unlikely. Swab **before** a cervical exam; sex in the last 24 hrs can cause a false positive.
- **Continuous fetal monitoring** and contraction pattern
- **Look for the cause:** urine culture, GBS and vaginal cultures, hydration

STEP 2 — Treat Early

- **< 34 weeks → betamethasone IM**, 2 doses 24 hrs apart, to mature baby's lungs
- **Treat the cause:** antibiotics for infection, IV fluids for dehydration
- **Contractions settle?** Home with teaching: pelvic rest, activity as ordered, return if symptoms come back
- **Case management** for stress, lost income, and child care needs

STEP 3 — Active PTL

- **Tocolytics** buy about **48 hours** so steroids can work
- **Magnesium < 32 weeks** protects baby's brain (lowers cerebral palsy risk)
- **IV penicillin G** if GBS is positive or unknown
- **Call the NICU team** and warm the delivery room

Tocolytics

Memory hack: think **MINT**

Drug	Watch for	Nurse must-know
Magnesium sulfate	Flushing, warmth, drowsiness. **Toxicity:** RR < 12, absent DTRs, urine < 30 mL/hr, confusion	Check RR, DTRs, urine output often. **Calcium gluconate** at bedside (antidote)
Indomethacin (NSAID)	Baby: early closure of the ductus arteriosus, low amniotic fluid. Mom: stomach upset	**Only < 32 weeks**, short-term (about 48 hrs). Give with food
Nifedipine (calcium channel blocker)	Low BP, headache, flushing, fast heart rate	Check BP. Caution with magnesium (adds to low BP and muscle weakness)
Terbutaline (beta-agonist)	Fast HR in mom and baby, tremors, high blood sugar, low potassium, fluid in lungs	**Hold if mom's HR > 120.** Short-term only

RED FLAGS · STOP AND CALL THE PROVIDER

Signs of **magnesium toxicity** · mom's **HR > 120**, chest pain, or shortness of breath on terbutaline · **crackles or dropping SpO_2** (fluid in the lungs) · **worrying fetal heart rate changes**

PRETERM NEWBORN: WHY COLD IS DANGEROUS

Thin skin, little fat → **heat loss** → Body burns **brown fat** to warm up → Uses up **oxygen + glucose** → **Breathing trouble + low sugar**

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The Preterm Newborn: Warm · Breathe · Feed · Rest

1. Warm

- **Pre-warm** the room and radiant warmer
- Radiant warmer or **incubator**, hat, skin-to-skin when stable
- Very preterm: **plastic wrap/bag** from the shoulders down, **without drying first**

2. Breathe

- Low **surfactant** → respiratory distress
- Watch for **grunting, flaring, retractions**, RR > 60
- Surfactant, **CPAP**/oxygen; **gentle** stimulation only

3. Feed

- Weak **suck-swallow-breathe** → start with **IV or gavage**
- **Ready to nipple?** Awake, rooting, gag reflex, coordinated suck-swallow-breathe
- Check **glucose**, I&O, **daily weight**; breast milk + lactation

4. Rest & Protect

- **Cluster care**, dim lights, quiet room
- Weak immune system → **strict hand hygiene**
- Expected: **lanugo**, thin see-through skin, floppy tone (**hypotonia**)

MUST-KNOW FOR THE NCLEX

- **Steroids before 34 weeks** are about baby's lungs; tocolytics just buy time for them to work.
- **On magnesium?** Check RR, DTRs, and urine output. Calcium gluconate is the antidote.
- **Negative fFN** is the reassuring result.
- **Preterm baby priorities:** warmth, breathing, feeding readiness, and rest.

QUICK CHECK · TEST YOURSELF

1. Contractions at 33 weeks? Teach the 4 S's: Stop, Side, Sip, and _____.
2. Give betamethasone before ___ weeks to help mature baby's lungs.
3. Name the 4 tocolytics using MINT.
4. A respiratory rate below ___ on magnesium means stop the infusion.
5. A preterm baby is ready to nipple feed when suck, swallow, and _____ are coordinated.

Answers: 1. Sixty (call if still contracting after 1 hr) · 2. 34 · 3. magnesium sulfate, indomethacin, nifedipine, terbutaline · 4. 12 · 5. breathing

For study use only. Always follow facility protocols, current guidelines, and provider orders.

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